



VIKENxx-07

Aetos Kenazor Cream 2% w/w

DESCRIPTION

White to off white, opaque cream.

COMPOSITION

Ketoconazole 2% w/w.

PHARMACODYNAMICS

Pharmacodynamic Properties

Pharmacotherapeutic group: Antifungals for Topical Use, Imidazole and triazole derivatives, ATC code: D01AC08.

Mechanism of Action:

Ketoconazole inhibits the biosynthesis of ergosterol in fungi and changes the composition of other lipid components in the membrane.

Pharmacodynamic Effects

Usually ketoconazole cream acts rapidly on pruritus, which is commonly seen in dermatophyte and yeast infections, as well as skin conditions associated with the presence of *Malassezia spp.* This symptomatic improvement is observed before the first signs of healing are observed.

Microbiology

Ketoconazole, a synthetic imidazole dioxolane derivative, has a potent antimycotic activity against dermatophytes such as *Trichophyton spp.*, *Epidermophyton floccosum* and *Microsporum spp.* and against yeasts, including *Malassezia spp.* and *Candida spp.* especially the effect on *Malassezia spp.* is very pronounced.

PHARMACOKINETICS

Plasma concentrations of ketoconazole were not detectable after topical administration of Aetos Kenazor Cream 2% w/w in adults on the skin.

INDICATIONS

Aetos Kenazor Cream 2% w/w is indicated for topical application in the treatment of dermatophyte infections of the skin: tinea corporis, tinea cruris, tinea manus and tinea pedis due to *Trichophyton rubrum*, *Trichophyton mentagrophytes*, *Microsporum canis* and *Epidermophyton floccosum*, as well as in the treatment of cutaneous candidosis and tinea (pityriasis) versicolor.

Aetos Kenazor Cream 2% w/w is also indicated for the treatment of seborrheic dermatitis, a skin condition associated with the presence of *Malassezia furfur*.

CONTRAINDICATIONS

Aetos Kenazor Cream is contraindicated in individuals who have shown hypersensitivity to ketoconazole and any of its ingredients.

WARNINGS AND PRECAUTIONS

Aetos Kenazor Cream 2% w/w is not for ophthalmic use.

If coadministered with a topical corticosteroid, to prevent a rebound effect after stopping a prolonged treatment with topical corticosteroids, it is recommended to continue applying a mild

topical corticosteroid in the morning and to apply Aetos Kenazor Cream 2% w/w in the evening, and to subsequently and gradually withdraw the topical corticosteroid therapy over a period of 2 – 3 weeks.

PREGNANCY AND LACTATION

Pregnancy

There are no adequate and well-controlled studies in pregnant women. Plasma concentration of ketoconazole are not detectable after topical application of Aetos Kenazor Cream 2% w/w to the skin of non-pregnant humans. There are no known risks associated with the use of Aetos Kenazor Cream 2% w/w in pregnancy.

Breast-feeding

There are no adequate and well-controlled studies in lactating women. There are no known risks associated with the use of Aetos Kenazor Cream 2% w/w in lactation.

MAIN SIDE / ADVERSE EFFECTS

The safety of ketoconazole cream was evaluated in 1079 subjects who participated in 30 clinical trials. Ketoconazole cream was applied topically to the skin. Based on pooled safety data from these clinical trials, the most commonly reported ($\geq 1\%$ incidence) adverse reactions were (with % incidence): application site pruritus (2%), skin burning sensation (1.9%), and application site erythema (1%).

Including the above-mentioned adverse reactions, the following table displays adverse reactions that have been reported with the use of ketoconazole cream from either clinical trial or postmarketing experiences.

System Organ Class	Adverse Reactions		
	Frequency Category		
	Common	Uncommon	Not Known
Immune System Disorder		Hypersensitivity	
Skin and Subcutaneous Tissue Disorders	Skin burning sensation	Bullous eruption Dermatitis contact Rash Skin exfoliation Sticky skin	Urticaria

1 2 5 3



Aetos Kenazor Cream 2% w/w

System Organ Class	Adverse Reactions		
	Frequency Category		
	Common	Uncommon	Not Known
General Disorders and Administration Site Conditions	Application site erythema Application site pruritus	Application site bleeding	
		Application site discomfort	
		Application site dryness	
		Application site inflammation	
		Application site irritation	
		Application site paresthesia	
		Application site reaction	

Treatment should be continued, until a few days after disappearance of all symptoms. The diagnosis should be reconsidered if no clinical improvement is noted after 4 weeks of treatment.

Special populations

Pediatrics

There are limited data on the use of ketoconazole 2% cream in pediatric patients.

Administration

Topical administration to the skin.

ROUTE OF ADMINISTRATION

For topical administration

EFFECTS ON ABILITY TO DRIVE AND USE MACHINE

Not Applicable

INSTRUCTIONS FOR USE

To open the tube, unscrew the cap. Then pierce the seal of the tube with the pin on the top of the cap.

Storage: Store below 30°C.

Presentation/Packing: 15g aluminium tube.

Product Registration Holder:

Abiotide Sdn. Bhd.

121, Jalan Tunku Abdul Rahman,
30010, Ipoh, Perak, Malaysia.

Manufactured by:

HOVID Bhd.

121, Jalan Tunku Abdul Rahman
(Jalan Kuala Kangsar),
30010 Ipoh, Perak, Malaysia.

Distributed by:

Aetos Pharma Sdn Bhd

No. 66B, Tingkat 2, Jalan Cerdas,
Taman Connaught, 56000 Kuala Lumpur, Malaysia.

Revision date: January 2024

DRUG INTERACTIONS

None known.

OVERDOSE

Topical Application

Excessive topical application may lead to erythema, edema and a burning sensation, which will disappear upon discontinuation of the treatment.

Ingestion

In the event of accidental ingestion, supportive and symptomatic measures should be carried out.

DOSAGE AND ADMINISTRATION

Dosage:

Cutaneous candidosis, tinea corporis, tinea cruris, tinea manus, tinea pedis and tinea (pityriasis) versicolor: it is recommended that Aetos Kenazor Cream 2% w/w be applied once daily to cover the affected and immediate surrounding area.

Seborrheic dermatitis: Aetos Kenazor Cream 2% w/w should be applied to be affected area once or twice daily.

The usual duration of treatment is: tinea versicolor 2 – 3 weeks, yeast infections 2- 3 weeks, tinea cruris 2 – 4 weeks, tinea corporis 3 – 4 weeks, tinea pedis 4 – 6 weeks.

The usual initial duration of treatment in seborrheic dermatitis is 2 to 4 weeks. Maintenance therapy is applied once or twice weekly in seborrheic dermatitis.